



## American Society of Radiologic Technologists

Robert F. Kennedy, Jr.  
Department of Health and Human Services  
ATTN: HHS Health Sector AI RFI  
Mail Stop: 7033A, 330 C Street SW  
Washington, DC 20201

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**Subject docket ID: (HHS-ONC-2026-0001; 90 FR 60108) HHS Health Sector RFI—Request for Information:  
Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care**

Dear Secretary Kennedy

The American Society of Radiologic Technologists represents more than 155,000 medical imaging technologists and radiation therapists across the nation. Our mission as the premier professional association for the medical imaging and radiation therapy community is to advance and elevate the medical imaging and radiation therapy profession and enhance quality, safe patient care. With that mission in mind, ASRT appreciates the opportunity to comment on the ASTP's request for information "Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care."

Artificial Intelligence has the potential to elevate patient outcomes, expand access to care, and improve efficiency. However, such implementation, especially within medical imaging and radiation therapy, must prioritize patient safety through clear, strong national governance.

AI can be a tool that augments clinical expertise but cannot be a substitute for judgement and experience of healthcare providers. Federal policy should ensure that AI deployment strengthens person-centered care while creating pathways for responsible adoption.

HHS must issue clear national guidance on AI governance and accountability within health systems, particularly for post-deployment use. Today, both clinicians and institutions face uncertainty about expectations for oversight, documentation, monitoring, and responsibility allocation, especially in multi-vendor environments where AI tools interact with EHRs, imaging systems, treatment planning systems, and delivery devices. This uncertainty slows adoption and increases risk. HHS can play a critical role by articulating baseline governance expectations applicable to both FDA-regulated AI devices and non-device clinical AI. Such guidance must affirm that all final decision-making remains with the appropriate licensed physician, and that AI technologies cannot override the medical imaging and radiation therapy professionals' expertise in accordance with the physician's orders and clinical indications. Expectations for transparent logging of AI recommendations, continuous performance monitoring, version and update management, and incident reporting must also be clearly established.

In conclusion, HHS must prioritize the critical safeguards that protect patients from inappropriate and dangerous care. Through strong, clear national governance on the reasonable adoption of AI, HHS will be able to balance the evolving nature of new technologies with a mandate to implement federal policy that prioritizes patient care

and access. Any adoption of artificial intelligence in clinical settings must be utilized as a tool within workflow and cannot be implemented as a replacement for clinical education and expertise.

ASRT would be honored to serve as a resource for questions related to AI in medical imaging and radiation therapy and look forward to working with HHS as implementation frameworks are developed.

We appreciate the opportunity to provide comments. Please do not hesitate to reach out me at [jbradley@asrt.org](mailto:jbradley@asrt.org) if you have any questions.

A handwritten signature in black ink, appearing to read 'J. Bradley', with a long horizontal flourish extending to the right.

Jason J. Bradley  
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